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THEORETICAL ARTICLE

Values in Acceptance and Commitment Therapy and Frankl's Categories of Values

Convergences and Differences Between Two Conceptions of Values

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ABSTRACT

Values occupy a central position in both acceptance and commitment therapy (ACT) and logotherapy. In ACT, values are understood as freely chosen, verbally constructed qualities of ongoing action that guide committed behavior and give direction to psychological flexibility. Viktor E. Frankl, in contrast, distinguished creative, experiential, and attitudinal values, which, in his view, are not invented but discovered in the respective situation. This theoretical paper systematically compares the two conceptions. It identifies convergences, such as the distinction between values and goals, the emphasis on action and engagement, the significance of values in dealing with unavoidable suffering, and the function of values as a counterweight to an exclusively symptom-oriented therapy. At the same time, it highlights fundamental differences that arise from distinct philosophical backgrounds: the functional contextualism of ACT on the one hand and Frankl's phenomenological and personalist anthropology on the other. Particular attention is paid to the question of whether values are chosen or discovered and to the status of attitudinal values. The paper concludes with reflections on how both traditions can learn from each other without blurring their respective premises.

Keywords: acceptance and commitment therapy · values · logotherapy · attitudinal values · psychological flexibility

1 Introduction

In the psychotherapy of recent decades, the concept of values has experienced a remarkable renaissance. A major contribution to this has been made by Acceptance and Commitment Therapy (ACT), which identifies values as one of its six core processes (Hayes et al., 1999, 2012). For readers familiar with logotherapy, this development is of particular interest, because Frankl had already developed, decades earlier, a theory of values that stands at the center of his psychotherapeutic approach (Frankl, 1946).

The obvious assumption that these are two variants of the same basic idea, however, falls short. The two traditions emerged in different epistemological and anthropological contexts, and the same terms sometimes carry different meanings in them. The present contribution compares the two conceptions in order to name commonalities precisely and to avoid prematurely leveling differences.

The comparison proceeds in four steps. First, the ACT concept of values is placed in its theoretical context, and Frankl's theory of values is then outlined. Building on this, convergences and differences are worked out. Finally, the implications for practice, assessment, and research are discussed. The contribution confines itself to the conceptual level and makes no statements about the comparative efficacy of the two approaches.

2 Values in Acceptance and Commitment Therapy

ACT understands itself as an application of functional contextualism and relational frame theory, a behavior-analytic approach to explaining human language and cognition (Hayes et al., 2001). According to this understanding, psychological suffering arises essentially from psychological inflexibility, that is, from the dominance of rigid verbal rules, experientially avoidant strategies, and a narrowing of the behavioral repertoire (Hayes et al., 2006). Psychological flexibility, conversely, is regarded as a fundamental aspect of mental health (Kashdan & Rottenberg, 2010).

In ACT, values are described as freely chosen, verbally constructed consequences of ongoing, evolving patterns of activity that function as reinforcers and give direction to action (Hayes et al., 2012). Essential is the distinction between values and goals: goals can be achieved and completed, whereas values resemble a compass direction by which one orients oneself without ever arriving. Values, moreover, are not supposed to be derived from duty, social pressure, or the wish to avoid pain but to be experienced as personally meaningful. Dedicated instruments have been developed to assess valued action, such as the Valued Living Questionnaire (Wilson et al., 2010).

In therapeutic work in ACT, values are frequently explored through experiential exercises, for instance by imagining one's own funeral or by asking what one wants to stand for in life (Hayes et al., 1999). Values clarification is not a one-time act but an ongoing process closely linked to committed action. At the same time, care is taken not to establish values as new rigid rules that could in turn lead to inflexibility.

3 Frankl's Categories of Values

Frankl distinguished three main ways in which people can realize meaning (Frankl, 1946, 1955). Creative values are realized in what a person gives to the world, for instance through work or creation. Experiential values are realized in what a person receives from the world, for instance in the encounter with nature, art, or another human being. Attitudinal values, finally, are realized in the stance a person takes toward an unalterable fate, in particular toward unavoidable suffering.

For Frankl, meaning and values are not invented but found. According to his understanding, meaning is always related to a specific situation and addresses itself as a call to the individual person, who responds to it in responsibility (Frankl, 1969). Conscience functions here as the "organ of meaning," which can sense the unique meaning of a situation. This conception is embedded in an anthropology that assumes a spiritual dimension of the human being and regards self-transcendence – the orientation toward something or someone beyond oneself – as the hallmark of human existence.

Frankl's categories of values are not to be understood in the sense of a rigid hierarchy; nevertheless, he attributed a special dignity to attitudinal values because they can still be realized even where creative and experiential possibilities are exhausted. Herein lies the point of his thesis of the unconditional meaningfulness of life: as long as a person lives, there remains at least the possibility of taking a stand toward one's fate (Frankl, 1946).

4 Convergences

Despite their different origins, several convergences can be identified. First, both approaches emphasize the distinction between an overarching direction and concrete goals. Frankl's talk of meaning as a task that arises anew in every situation and the ACT metaphor of the compass direction both point to the fact that values cannot be checked off. Second, both traditions place the emphasis on action and commitment rather than on mere insight. Third, both see values as a counterweight to a therapy oriented exclusively toward symptom reduction.

Most striking is their closeness in dealing with unavoidable suffering. ACT invites people not to fight painful inner experiences but to accept them in the service of value-guided action. Frankl's attitudinal values aim at a related stance: where a situation cannot be changed, the freedom to take a stand toward it remains. In both cases, coping with suffering is understood not as its elimination but as a change in one's relationship to it.

5 Differences

The most fundamental difference concerns the status of values. In ACT, values are chosen; their significance results from their function for a person's action, and functional contextualism deliberately refrains from ontological statements about whether values exist independently of human beings (Hayes, 2004). For Frankl, by contrast, values and meaning cannot be chosen arbitrarily but have a demand character that confronts the person from outside. This difference is not merely terminological: it has consequences for the question of whether a therapist may question a client's values, and for the significance of responsibility and conscience.

A second difference concerns attitudinal values. In ACT, acceptance is a process that enables value-guided action but is not itself a value. For Frankl, on the other hand, the stance toward suffering can itself be meaningful – even when no further possibilities for action exist. This position gains particular weight in situations in which the scope for action is severely limited, for instance in serious illness at the end of life.

A third difference concerns the role of self-transcendence. With self-as-context, ACT has a perspective that leads beyond identification with self-

concepts. Frankl's self-transcendence, however, means something else: turning toward a meaning or another person beyond one's own experience. Existential analysis, too, has emphasized personal stance-taking as an independent dimension (Längle, 2013).

A fourth difference concerns the relationship between language and meaning. In ACT, language is regarded as double-edged: the ability to form relational frames makes values and future orientation possible but is at the same time a source of rumination, self-evaluation, and experiential avoidance (Hayes et al., 2001). Frankl, by contrast, considers the spiritual dimension not primarily in terms of its risks but as a source of freedom and responsibility. ACT's skepticism toward the dominance of verbal rules has no direct counterpart in logotherapy, although Frankl's critique of hyperreflection points in a related direction.

6 Conclusions

The comparison shows that ACT and logotherapy are astonishingly close in their practical orientation but diverge clearly in their philosophical foundations. For practice, this can be enriching: ACT offers a multitude of tested exercises for clarifying values and fostering psychological flexibility; logotherapy offers a differentiated language for dealing with unalterable suffering and for the question of what a person orients themselves toward beyond themselves. For research, the task arises of operationalizing the differences in the conceptions of values in such a way that their respective contributions to change become empirically distinguishable. An eclectic blending without reflection on the premises, by contrast, would serve neither tradition.

Different requirements arise from this for measurement. Instruments from the ACT tradition typically assess how important certain life domains are to a person and how consistently they act in accordance with them (Wilson et al., 2010). Logotherapeutic concepts such as the realization of attitudinal values can be represented only inadequately with such instruments, since they concern precisely those situations in which value-consistent action in the usual sense is hardly possible. Here there is a need for further development in assessment.

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